

Aura Clinical — Healthcare Attorney Consultation Intake

Date prepared: 2026-05-25 **Prepared by:** Kearney Adams Jr., founder, Aura Protocols LLC **Purpose:** Maximize a 1–2 hour paid consult with a telehealth-specialized healthcare attorney. The goal of the engagement is a go/no-go signal on launching a peptide-focused telehealth business in a single US state, plus a concrete entity-structure recommendation.

1. About me

- Solo non-technical founder. Full-time on this project.
 - Bootstrapped — total runway for legal, entity, insurance, and pre-launch infrastructure is approximately **\$8,000–20,000 over the next 4–6 months**.
 - I already own and operate `shop.auraprotocols.com`, a Next.js affiliate site for research-grade peptides. It is currently pre-revenue. The site links to third-party vendors (Core Peptides, Limitless Life Nootropics, Swiss Chems, etc.) under "research use only" disclaimers and FTC affiliate disclosures. I do not handle product or take payment for product.
 - **Aura Protocols LLC** is already formed (LegalZoom). It owns the affiliate site and will own the upcoming "Engine" biometric product (see §2.2). This is the entity I currently operate under.
 - I have **not** yet formed any clinical/medical entity, signed any pharmacy or MD agreement, taken on any patient, or made any prescribed-medication claim. I am at the pre-formation stage of the clinical side and want legal direction before spending.
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2. The business I am building

2.1 The three layers

I am building **one combined company** with three customer-facing surfaces:

Layer	Surface	Role	Monetization
Editorial	<code>auraprotocols.com</code> (planned: collapse <code>shop.auraprotocols.com</code> into apex)	SEO/education entry point. Reviews, comparisons, peptide dosing guides.	Affiliate commissions on research-grade vendor links (third-party).

Layer	Surface	Role	Monetization
Engine	Same site as Editorial, dynamic routes (/connect, /recommend).	Free biometric-driven peptide protocol recommender. User connects Whoop/Oura/Apple Health via Terra API aggregator; we return a personalized protocol suggestion.	None directly — converts qualified leads downstream.
Clinical	auraclinical.com (or similar TBD) — separate brand, separate LLC, separate domain, separate payment processor, separate analytics	Vertical telehealth. Patient intake → licensed MD review → prescribed compounded peptides → recurring subscription.	\$250–500/mo subscription per patient. This is the primary revenue driver.

2.2 Funnel logic

- 1 User finds Editorial via SEO (searches for peptide info, dosing, vendor comparisons).
- 2 CTAs route them to the Engine to "get a personalized protocol."
- 3 Engine outputs a recommendation. Every recommendation surface shows two CTAs:
 - **"Get this prescribed at Aura Clinical →"** (primary — routes to the Clinical brand)
 - **"Buy research-grade vials →"** (secondary — third-party affiliate links, "research use only" disclaimers)
- 1 Clinical path: async intake form (pre-populated with Engine output), state-licensed MD reviews, compounding pharmacy ships, recurring monthly subscription.

2.3 Intended formulary at Clinical launch

I am aware that the FDA enforcement posture toward compounded peptides has tightened materially in 2024–2025 (CJC-1295 Cat 2 listing, semaglutide shortage exemption sunset, increased 503A inspections). I want the attorney's read on what is **realistically prescribable and compoundable today** in our launch state from the following candidate list:

- Sermorelin (GH-releasing)
- Compounded semaglutide / tirzepatide (where still allowed)
- NAD+ injections
- Testosterone (HRT)

- Oxytocin
- Sildenafil / tadalafil (compounded oral troches)
- BPC-157, TB-500, CJC-1295, Ipamorelin — I understand these are likely **not** safely prescribable through a 503A pharmacy partnership in today's environment, but want the attorney's confirmation and any nuance per state.

2.4 Launch posture

- **Single launch state:** considering Florida, Texas, or Arizona. Open to attorney's recommendation based on telehealth practice acts, corporate practice of medicine rules, formulary tolerance, and patient demand density.
- **Async-first** (questionnaire + asynchronous physician review), with synchronous video on request or when state law requires.
- **Subscription billing** via Stripe (telehealth-legal use case, distinct from the affiliate side).
- **Vendor-platform UI** (Healthie, Spruce Health, or similar) rather than custom EHR. Will brand the platform.
- **MD network** via Wheel, SteadyMD, OpenLoop, or similar — or direct contract with a single licensed MD in the launch state.
- **Compounding pharmacy partner** TBD — discovery calls planned with Empower (TX), Olympia (FL), Tailor Made (KY).

2.5 The relationship between Aura Protocols LLC (affiliate + Engine) and Aura Clinical (telehealth)

- I intend to keep these as **two separate LLCs**.
 - The Editorial/Engine side links users to the Clinical brand via clear "get this prescribed at our sister brand" handoffs, similar to how Hims/Hers operate their content properties.
 - I do not want a category-level adverse event on either side to be able to take down the other.
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3. Questions for the attorney

3.1 Entity structure & corporate practice of medicine

- 1 In our recommended launch state (FL / TX / AZ), what is the correct legal structure?
 - Single LLC?
 - LLC + Professional Corporation (PC) with a Management Services Organization (MSO) agreement?
 - Other?

- 1 Who must own the PC (if required)? Must it be a licensed physician in the same state? What is the standard MSO arrangement that lets a non-licensed founder operate the business side legally?
- 2 What is the realistic all-in cost for entity formation, operating agreements, MSO agreement, and registered agents?
- 3 Are there states where the corporate practice of medicine doctrine is loose enough that a regular LLC works, and would you recommend any of those over the three I've named?

3.2 Telehealth practice act — launch state

- 1 Does the launch state allow **async-only** patient-physician relationship establishment, or is initial synchronous (video) contact required?
- 2 Is there an in-state physical address / "physician establishment" requirement?
- 3 What are the minimum documentation, identity-verification, and informed-consent requirements?
- 4 Any state-specific advertising or disclosure rules I need to know about (e.g., Florida required disclaimers)?

3.3 Formulary and FDA enforcement reality

- 1 Given the current FDA enforcement posture, which of the candidate substances in §2.3 are realistically prescribable through a 503A compounding pharmacy in our launch state, today?
- 2 What is your read on the BPC-157 / TB-500 / CJC-1295 / Ipamorelin path — is there any compliant route, or should I treat these as not available regardless of demand?
- 3 What is the current status of compounded semaglutide / tirzepatide for non-shortage prescribing, and how does that interact with state pharmacy board rules?
- 4 Are there compounded-peptide categories I'm not considering that you've seen work well for similar telehealth startups (e.g., specific GLP-1 analogs, growth factor blends, sermorelin combinations)?

3.4 MD network and physician relationship

- 1 We are deciding between (a) contracting a single licensed MD in the launch state, vs (b) using a telehealth MD network platform (Wheel, SteadyMD, OpenLoop). What are the legal differences, particularly around anti-kickback statutes, fee-splitting prohibitions, and the patient-physician relationship?
- 2 If we use a network platform, do we need a separate Medical Director relationship with a single state-licensed MD anyway?
- 3 What is the typical compensation structure — per-consult fee, salary, equity — that does not violate fee-splitting rules?
- 4 What liability flows back to me / the LLC vs. stays with the prescribing MD?

3.5 Pharmacy partnership

- 1 What contractual terms should I require from a 503A compounding pharmacy partner to insulate Aura Clinical from a pharmacy-side regulatory action?
- 2 Are there pharmacy-side red flags I should screen for (recent FDA warning letters, state board actions, formulary restrictions, ownership structure)?
- 3 Is the "pharmacy ships direct to patient" model legally cleaner than "pharmacy ships to clinic, clinic re-ships"?

3.6 Patient relationship establishment & ongoing care

- 1 What does a defensible initial-consult workflow look like for an async-first telehealth (questionnaire content, supporting documents required, MD review documentation)?
- 2 What follow-up cadence is legally adequate — quarterly check-ins, annual labs, asynchronous messaging only?
- 3 What records-retention obligations apply, and how should the EHR platform support them?

3.7 Marketing, advertising & FTC/FDA claims

- 1 Given the affiliate side (Aura Protocols LLC) uses content like "best peptides for weight loss" and "vendor X vs vendor Y" comparisons, can the Editorial site link to Aura Clinical CTAs without creating a conflict of interest disclosure problem or an off-label promotion issue?
- 2 What efficacy, performance, or anti-aging claims can Aura Clinical make in its own marketing without triggering FTC or FDA action?
- 3 What disclaimers must appear on the Engine output (which is a biometric-driven *suggestion* engine, not a diagnosis) and on Clinical marketing pages?
- 4 Can the Engine output legally pre-populate intake forms used by the prescribing MD? Does that create a legal-practice-of-medicine question for the software?

3.8 Data, HIPAA, and the Engine/Clinical boundary

- 1 The Engine collects wearable fitness data (HRV, sleep, recovery, steps). It does **not** collect medical history or diagnoses. **Is this PHI or fitness data**, and does HIPAA apply to the Engine side, the Clinical side, or both?
- 2 When a user clicks "get this prescribed" and the Engine output is handed off to the Clinical intake, at what point does the data become PHI and trigger HIPAA?
- 3 Recommended technical and legal controls at that boundary (BAAs with which vendors, what to log vs not log, breach notification readiness)?
- 4 We will not use Meta Pixel or untreated GA on Clinical pages. Anything else we must positively exclude?

3.9 Affiliate site / Clinical brand separation

- 1 Same founder, same employees (just me), separate LLCs, separate brands, linked via "sister brand" CTAs. Is that enough legal separation to prevent a regulatory action against one from reaching the other?
- 2 Should there be an arms-length agreement between Aura Protocols LLC and Aura Clinical LLC governing the lead-handoff and any compensation between them?
- 3 Anti-kickback risk: if Aura Protocols LLC drives leads to Aura Clinical, and Clinical pays Aura Protocols a per-lead fee or revenue share, does that violate any federal or state anti-kickback / patient-brokering laws?

3.10 Insurance

- 1 What insurance coverage is non-negotiable at launch (entity malpractice, general liability, cyber/HIPAA breach, D&O)?
- 2 Recommended brokers or carriers for a single-state telehealth at this scale?
- 3 Approximate cost ranges for the policies above?

3.11 Multi-state expansion

- 1 What is the right legal pattern to expand from one launch state to multi-state — same LLC + new state foreign registrations, or new PC per state?
 - 2 What are the high-cost / high-friction states I should defer indefinitely?
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4. What I want to walk out of the consult with

A clear, written recommendation on:

- 1 **Entity structure** for the launch state (LLC, LLC+PC+MSO, or other), with estimated total formation cost.
 - 2 **Go/no-go on the candidate launch states** (FL, TX, AZ — or attorney-recommended alternative).
 - 3 **Realistic launch formulary** given current FDA/state pharmacy board enforcement.
 - 4 **MD network model** (platform vs direct hire) given anti-kickback and fee-splitting rules.
 - 5 **Affiliate-to-Clinical lead handoff structure** that does not trigger anti-kickback / patient-brokering exposure.
 - 6 **List of next legal deliverables** (operating agreements, BAAs, patient consent forms, marketing disclaimers) and rough cost to produce them.
 - 7 **Any deal-breaker** I should know now before spending another dollar.
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5. Out of scope for this consult

- IP / trademark strategy (separate matter, lower priority).
- Employment law (no employees planned at launch).
- Tax structure (will engage a CPA separately).
- Detailed contract drafting (this consult is strategy + structure; drafting is a follow-on engagement).